Market Access Insight: Ocrevia in Spain (2023)

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Access Overview

Reimbursement for Ocrevia in Spain was approved on a managed entry agreement requiring real-world evidence submission within 18 months of launch under AEMPS.

The regional national health service system applies health technology assessment before listing Ocrevia for multiple sclerosis.

Payer Perspectives

Payers in Spain view the price of Ocrevia as acceptable only with a demonstrated clinical benefit versus the standard of care.

Formulary placement is reviewed annually, and pricing is linked to outcomes in the registry.

Sequencing after progression remains an open question; neurologists in Spain reported using a different mechanism of action after Ocrevia fails.

Payer decisions increasingly rely on health technology assessment, requiring evidence of clinical benefit versus the standard of care and cost-effectiveness analyses.

Education needs include biomarker testing, management of adverse events and counselling patients on the expected benefit-risk.

Respondents expect real-world persistence on Ocrevia to be lower than in trials, mainly because of monitoring requirements and the complexity of managing infusion reactions.

Guideline committees are expected to review the evidence base for multiple sclerosis again next year, which could change eligibility criteria and referral patterns.

Access to specialist centres influences adoption: academic centres adopt new therapies faster than community practices, where formulary restrictions and reimbursement criteria apply.

Peer-to-peer education and congress data were cited as the most trusted sources when neurologists evaluate new evidence on Anti-CD20 antibody agents.

Treatment burden, including monitoring visits and monitoring requirements, was described as a key factor in long-term adherence and persistence.

Combination therapy and sequencing strategies remain debated; respondents noted limited head-to-head evidence between Ocrevia and competing agents.

Regional variation within Spain is notable: large centres report faster uptake and broader use than smaller regional hospitals.

Clinical guidelines in Spain recommend second-line use of agents in this class, and recent updates have broadened eligibility based on biomarker status and comorbidity profile.

Biosimilar and generic competition in adjacent classes is shaping price expectations, even where Ocrevia has no direct substitute.

Safety monitoring focused on constipation and infusion reactions, with dose interruption or discontinuation used for severe adverse events.

Evidence Requirements

Required evidence includes overall survival or response rate data, patient-reported outcomes and comparative effectiveness against Ocrevia's main competitor.

Subgroup analyses suggest that benefit with Ocrevia is most pronounced in patients with a favourable biomarker profile and a lower comorbidity burden.

Safety monitoring focused on ARIA imaging findings and constipation, with dose interruption or discontinuation used for severe adverse events.

Dose modification guidance for Ocrevia is generally followed, but neurologists said that practical monitoring of ARIA imaging findings depends on local capacity and patient travel distance.

The competitive set is evolving as new mechanisms enter phase 3 development, with readouts expected over the next two to three years.

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Education needs include biomarker testing, management of adverse events and counselling patients on the expected benefit-risk.

Respondents expect real-world persistence on Ocrevia to be lower than in trials, mainly because of injection-site discomfort and the complexity of managing constipation.

Medical education priorities include recognising ARIA imaging findings early, communicating realistic expectations, and coordinating with primary care.

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Cost-effectiveness submissions typically model first-line use, and payers question the durability of benefit beyond the trial follow-up period.

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Pricing and Reimbursement Outlook

A confidential rebate is expected to remain in place until 2025. Market access risk is high for new indications.

Neurologists emphasised that patient-reported outcomes and quality of life are increasingly important when choosing between Anti-CD20 antibody options.

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Medical education priorities include recognising headache early, communicating realistic expectations, and coordinating with primary care.

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Registry data show that outcomes in multiple sclerosis depend heavily on early diagnosis, access to specialist care and adherence to maintenance therapy.

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